

CLINICAL REPORT: DIGITAL RADIOGRAPHIC CHEST X-RAY FINDINGS (PA VIEW)

Patient Name:	Mr. Rajesh Kumar	UHID / Reg No:	FH-2026-884910
Age / Gender:	54 Yrs / Male	Examined Date:	May 17, 2026 02:30 PM
Referred By:	Dr. A. K. Sharma, MD	Department:	Pulmonary Medicine OPD

CLINICAL HISTORY & INDICATIONS

Patient presents with a persistent productive cough lasting for three weeks, continuous low-to-medium-grade fever fluctuating between 100.5°F and 101.8°F, localized right-sided pleuritic chest pain during deep inhalation, and progressive exertional fatigue. Patient reports a history of chronic tobacco smoking spanning 25 years.

RADIOGRAPHIC EXAMINATION FINDINGS

- Lung Parenchyma:** A prominent, dense, irregular radio-opacity/consolidation measuring approximately 4.2 cm is clearly visualized in the right upper lobe zone. There are additional patchy non-homogeneous infiltrates present bilaterally in the bases of both lungs, highly indicative of local air-space disease.
- Pleural Spaces:** Blunting of the right costophrenic angle is noted, consistent with scant localized pleural effusion on the right side. Left pleural space appears unremarkable.
- Mediastinum & Hilar Architecture:** Visible widening of the superior mediastinum with structural prominence and enlargement of the right hilar lymph nodes. The cardiac silhouette shows mild enlargement (cardiomegaly) with a left ventricular dominance pattern.
- Trachea:** The thoracic trachea is clear but exhibits a very mild leftward shift, likely secondary to a minor structural mass-effect from the right upper lobe consolidation.

RADIOLOGICAL IMPRESSION / INTERPRETATION

- **Large consolidation/mass**** in the right upper lobe. Differential diagnoses include primary active bacterial lobar pneumonia with necrotizing changes, or a primary endobronchial neoplastic mass lesion given the extensive history of tobacco consumption.
- **Right-sided scant pleural effusion**** with mild bilateral pulmonary venous congestion.
- **Right hilar lymphadenopathy**** and accompanying cardiomegaly.

CLINICAL RECOMMENDATION MATRIX

CLASSIFICATION: MEDIUM TO HIGH RISK STAGE. This condition is progressive and warrants prompt diagnostic escalation. It is highly recommended to perform a Contrast-Enhanced Computed Tomography (CECT) scan of the chest and abdomen immediately to further differentiate between neoplastic mass and infectious consolidation. Expeditious pulmonology consultation is advised.

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